



Suicide

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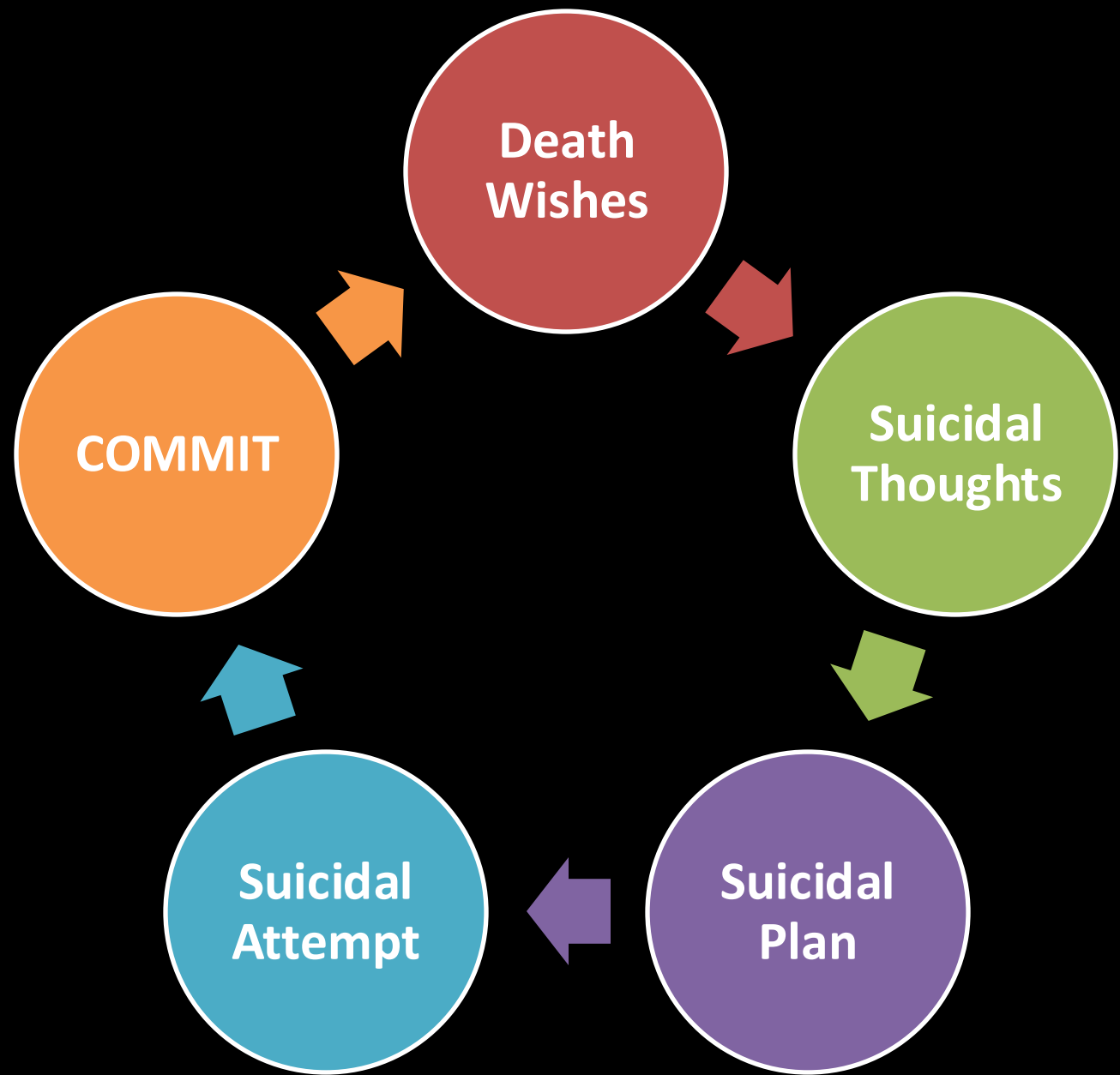


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Introduction

- Suicidal behaviour is a complex phenomenon across all age groups
- Aetiology is multifactorial and multidimensional



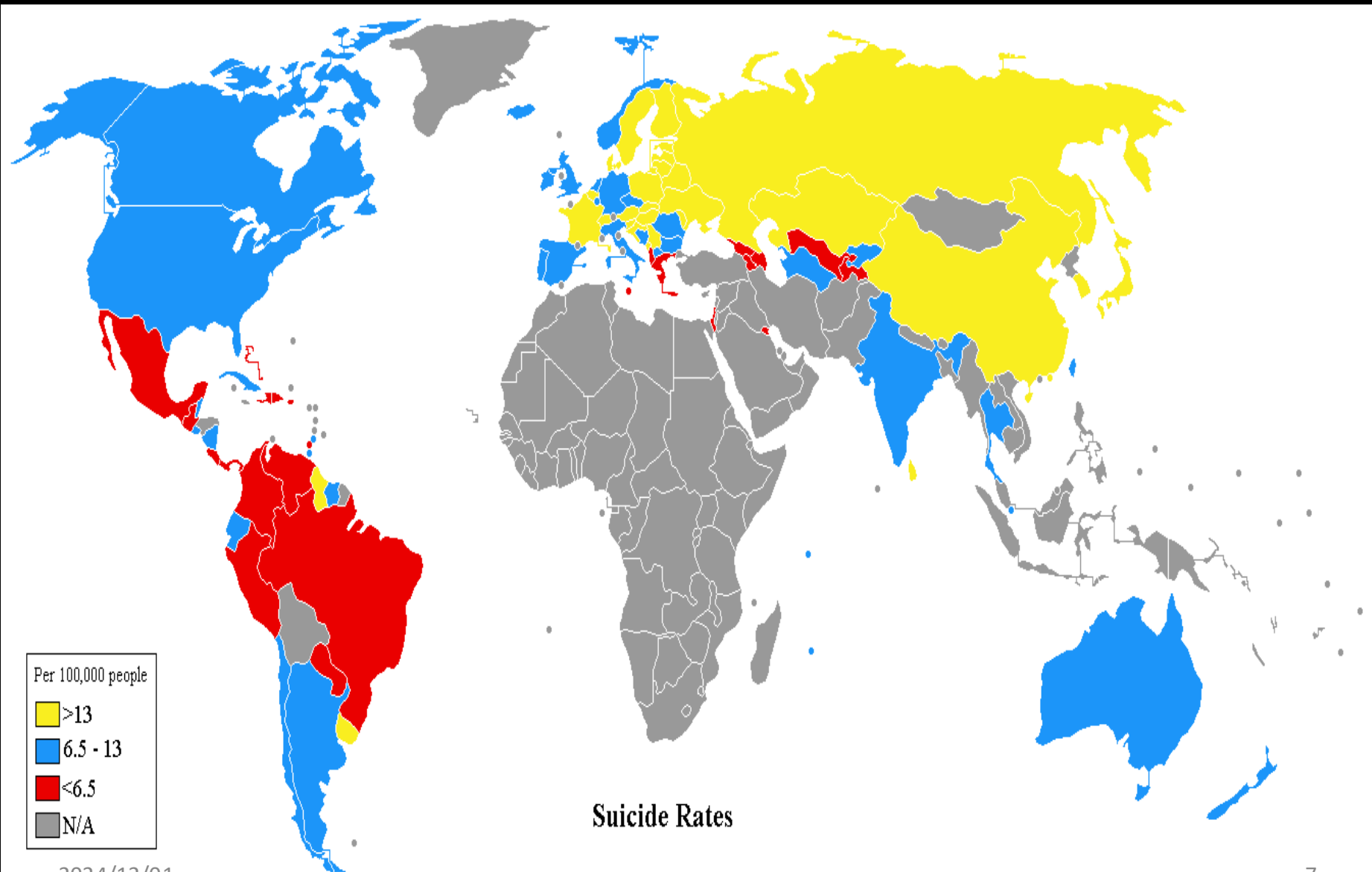
Global suicide rates

- **1 million** people die annually – suicide
- Globally suicide : 14-16 deaths/100 000/year
- One death by suicide every **40 seconds**
- 20-40 failed attempts per suicide

By 2020

- **1.53 million** suicides
- Worldwide: one death every **40 seconds**
- One suicidal attempt every **1-2 seconds**
- **60%** rise in suicide rates over the last 50 years

- Recently studies: **younger > older**
- **5-44** yrs account for **55%** of suicides
- Global incidence < 15 yrs doubled since 1960
- Global suicides: **male > female 3 : 1**



- **2nd** leading cause of death in the young
- **9.5%** non-natural deaths in young people
- **47.64%** of suicides in 20-34 yr
- **10%** in the 10-19 yrs
- Suicide attempts in the young : **females > males**

- **Peak time for suicides in young people:**
 - 06h00- 20h00
 - mostly over weekends
 - towards the end of the year
 - examination pressures, are high-risk periods for suicide behaviour in the young
- **Leading choice of method in young**
 - hanging
 - poisoning
 - firearms
 - gassing
 - fenestration (jumping from high places)
 - self burning???



Contact with Health Services

- $\frac{1}{2}$ suicide victims had contact with mental health services in the week before their death
- $\frac{1}{5}$ in the previous 24 hours
- $\frac{2}{3}$ saw their GP in the last month.

- $\frac{2}{3}$ of people who engaged in suicidal behaviour communicated their intent in preceding **3 months**

OR

- Consulted their GP for treatment for a psychological disorder at least **2 weeks** before the suicide act.

Factors associated with an increased risk of suicide

- Demographic
- Social factors
- Familial and Biological factors
- Physical illness
- Mental Illness & Psychological factors

SUICIDE: A MULTI-FACTORIAL EVENT



Demographic

Male

Younger > Elderly

Divorced, single or widowed

Socially isolated/living alone

Certain Professionals:

prostitutes
veterinary surgeons
pharmacists
farmers
doctors



Social Factors

- Social deprivation & social fragmentation
- poor economic conditions – unemployment
- Childhood adversity
- Interpersonal loss & conflict
- recent migration
- Financial difficulties.
- Very high social classes



Familial and Biological Factors

- **Family history of suicide - genetic risk**
- **Non-genetic: childhood abuse or neglect**
- **Reduced serotonin & 5-HIAA**
- **Reduced serum cholesterol** (Horton et al 1995)

Physical illness

- Chronic and severe physical illness.
- Cancer 2x suicide rate
- Epilepsy 5X suicide rate
- Chronic pain
- HIV/AIDS



Mental Illnesses

- Majority suicide victims have 1 or more psychiatric disorders
- **22%** suicides - in the first year of a mental illness
- Risk of suicide is high following discharge:
 - **25%** of post discharge suicides in the **first 3 months** most in the first **2 weeks** post discharge.

Rates of suicide for psychiatric disorders

- Major depression - **20 X**
- Elderly depressed - **35 X**
- Bipolar affective disorders - **15 X**
- Personality Disorders – **7X**
- Schizophrenia lifetime risk: **10%**

(Harris & Barraclough 1997)

Deliberate Self Harm

- Previously attempted suicide – Risk **↑ 38 X**
- Greatest risk suicide after act of DSH is in first 3 years especially in first 6 months
- **1%** DSH kill themselves in the next year.
- **15%** of DSH eventually kill themselves.

Psychological Factors

- **Hopelessness**
- **Impulsivity**
- **Dichotomous thinking**
- **Poor problem solving skills**

Suicide & Substance Misuse

- Opioid abusers: Risk of suicide ↑ 14 X
- Prescription drug abuse: ↑ 20 X
- Cannabis uses: ↑ 4 X
- Alcoholism: 15% risk of suicide

Protective Factors

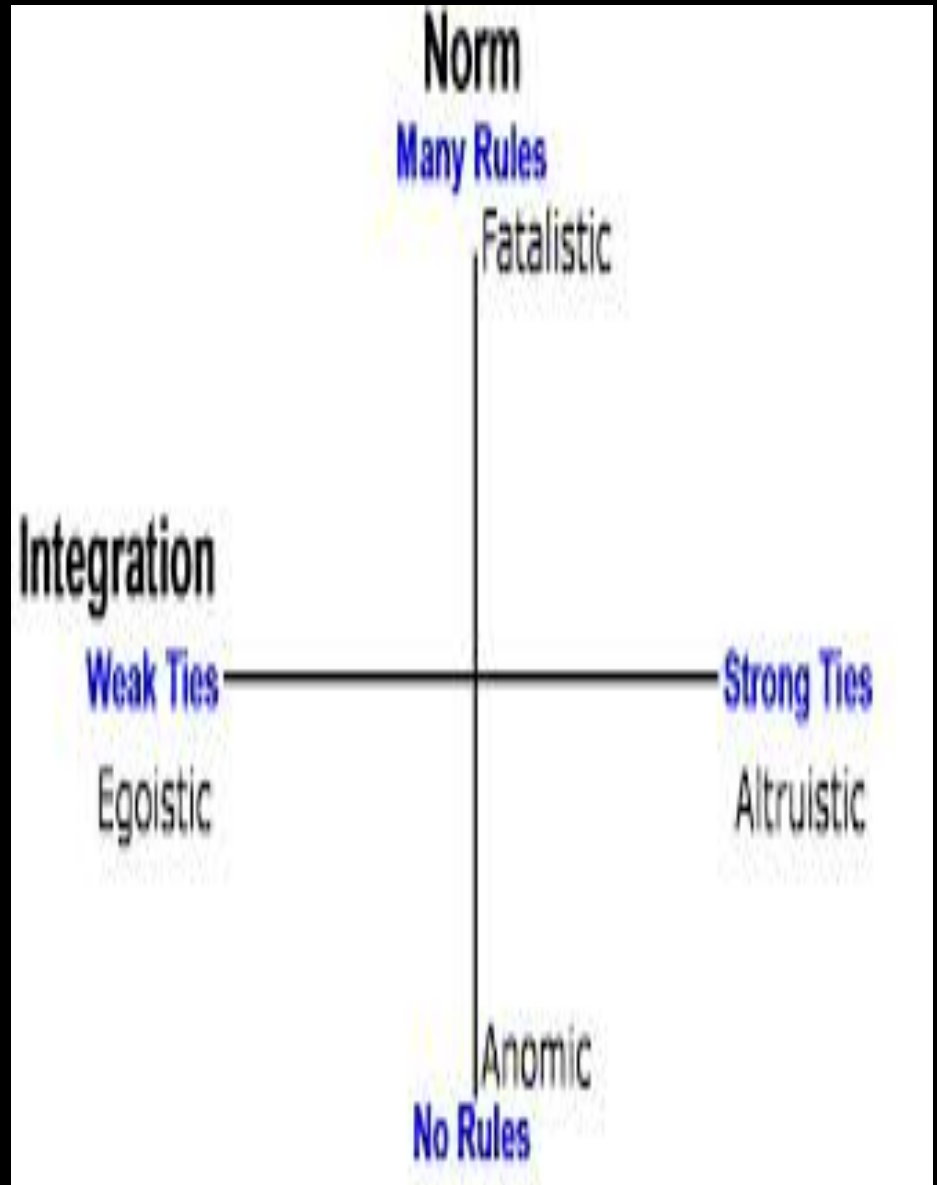
- Strong religious affiliation
- Married , children at home, pregnancy
- Responsibility to family
- Being optimistic



- **Fear of suicide**
- **Fear of social disapproval**
- **Moral objection to suicide**
- **Coping beliefs & positive coping skills**
- **Good social support**



Émile Durkheim



Predicting Suicides

- We cannot predict who will commit suicide.
- Determine individual's overall level of risk by:
 - identify the risk factors for suicide
 - the protective factors
 - likelihood of attempting suicide in the near future.
- Devise a treatment plan

Suicide Risk Assessment

Static

Dynamic

Stable

Future

(Bouch and Marshall 2005)

- **Static factors**
 - Fixed
 - Historical and demographic risk factors
 - e.g. previous history of suicide attempts
 - family history of suicide
 - male
- **Stable risk factors:**
 - may endure for many years
 - e.g. personality disorders
- (Bouch and Marshall 2005)

Dynamic risk factors

- present at particular times
- influenced by internal and external factors
- fluctuate in duration and intensity
 - e.g. Compliance ; hopelessness and suicidal ideation

Future risk factors

- could be anticipated
- arise from a change in circumstances
 - e.g. Gaining access to a preferred method of suicide
 - future contact with services
 - future response to treatment
 - future exposure to stress.

Management of a suicidal patient

- **Ensure patient is safe and medically fit**
- **Interview: tactful and sensitive**
- **Establish a rapport**
- **Gain the patient's trust**

- **Collateral information**
- **Review previous records**
- **Previous psychiatric Hx & Hx DSH**
- **Physical examination and Investigations**
- **Physical health: - chronic painful conditions**

What to focus on in the interview

- Triggers & motives for suicide (psycho-social stressors)
- DSH - impulsive or planned
 - What were the thoughts prior, during and after the act
 - patient's belief about the lethality of the method used.
- Current mental state
 - symptoms of mental illness
 - hopelessness & helplessness
 - Psychosis e.g: command hallucinations
 - current suicidal ideation, intent and plans.
 - Homicidal intent.

A high degree of suicide intent

- **The act was planned and prepared**
- **Precautions were taken not to be found**
- **A dangerous method was used**
- **Did not seek help after the act**
- **Left a will or suicide note or put affairs in order.**

Short Term Management

- Risk assessment → management plan.
- Ensure the patient's safety & alleviate distress
- The risk can be reduced by:
 - removing the means
 - reducing the opportunity
 - treating any associated illnesses

- **If the patient is high risk → admit to hospital.**
- **If patient refuses → admit as an involuntary patient under the Mental Health Care Act.**

Outpatient Treatment

patient is less risky

good social support

Carers can provide the appropriate level of supervision

They can obtain help in case of an emergency

- Seen frequently - first follow up within the first week
- Regular reviews of the suicidal risk and mental state.
- Prescribing medication: fewer side effects
less dangerous in an overdose
smaller quantities

Instilling Hope

- Identify positive reasons for remaining alive
- Guide the patient to a more positive view of the future

Suicide.

It's the one mistake you
won't live to regret.

THANK YOU